Report Analysis for St. Mary’s Hospital Group (Jan 2024 – Dec 2025)

Findings:

Revenue Performance

1. The healthcare group generated a total revenue of £6,454,834 during the reporting period. Although revenue performance was relatively strong, it fell short of the planned target of £7,000,000, achieving 92.21% of the expected revenue. This indicates that the organization was able to generate substantial income but did not fully meet its financial objective.

Patient Admissions

2. Total patient admissions reached 500, representing 76.92% of the planned target of 650 patients. This indicates that patient volume was lower than expected and may have contributed to the revenue shortfall observed during the reporting period.

Length of Stay

3. The average patient length of stay was 10.71 days, more than double the planned target of 5 days. This suggests potential inefficiencies in patient flow, discharge planning, treatment coordination, or administrative processes.

Patient Satisfaction

4. The average patient satisfaction score was 5.31 out of 10, achieving only 75.89% of the desired target. This indicates opportunities for improvement in patient experience and service quality.

Diagnosis Performance

5. Asthma emerged as the highest-performing diagnosis category, generating the highest number of admissions (65 patients) and the highest revenue contribution of approximately £805,568. This indicates that Asthma-related treatments represented a significant source of healthcare activity and revenue generation during the reporting period.

Recommendations

1. Management should investigate the factors contributing to the revenue shortfall and identify opportunities to increase service utilization. Particular attention should be given to high-performing diagnoses and cities to determine whether successful practices can be replicated across lower-performing areas.
2. Management should review patient acquisition, referral partnerships, and community outreach programs to increase admissions and improve healthcare service utilization.

3. Management should review discharge procedures, treatment workflows, and administrative approval processes to identify factors contributing to prolonged patient stays.
4. Regular patient feedback reviews should be conducted to identify service gaps and implement initiatives aimed at improving communication, responsiveness, and overall patient experience.
5. Management should examine the operational practices and treatment pathways associated with Asthma care to identify opportunities for improving performance across other diagnosis categories.

Further Investigatory Analysis

Findings:

Length of Stay vs Revenue

1. Further analysis was conducted to understand the relationship between patient Length of Stay and revenue generation. The Long Stay category (11–14 days) generated the highest total revenue of approximately £2.57 million and recorded the highest number of admissions (198 patients). Although Extended Stay patients remained hospitalized for a longer period, they generated lower total revenue (£1.82 million) than the Long Stay category. Additional analysis revealed that revenue per patient was slightly higher among Short and Moderate Stay patients than among Long and Extended Stay patients. This suggests that higher revenue was primarily driven by patient volume rather than increased revenue generated by individual patients.

Inter-City Performance Analysis

2. Further investigation revealed that London generated the highest overall revenue (£793,969), largely driven by higher patient admissions (65 patients) and the highest number of critical cases (19 patients). However, when revenue efficiency was assessed using revenue per patient, Sheffield (£13,897), Cardiff (£13,776), Glasgow (£13,410), and Nottingham (£13,292) outperformed London (£12,215). Cardiff was particularly noteworthy, achieving one of the highest revenue-per-patient figures while maintaining one of the shortest average lengths of stay (9.78 days). Despite this efficiency, Cardiff recorded the lowest overall revenue (£509,723) due to having the lowest patient volume (37 admissions). These findings suggest that city performance is influenced not only by patient volume but also by operational efficiency, case severity, and resource utilization.

Age Group vs Severity Status

3. Age-group analysis revealed that Adults accounted for the largest patient population (211 patients) and recorded the highest number of High-severity cases (61 patients). However, Elders recorded the same number of Critical cases (41 patients) despite having a significantly smaller patient population (157 patients). This suggests that older patients may be more vulnerable to severe medical conditions and require greater healthcare attention and resource allocation.

Payment Method Analysis

4. Payment method analysis revealed notable differences in patient outcomes and resource utilization. Patients covered under Private payment plans recorded the longest average length of stay (11.54 days) and the lowest satisfaction score (4.87), while Employer-sponsored patients generated the highest average revenue per patient (£13,696) and achieved one of the highest satisfaction scores (5.97). NHS patients recorded the shortest average length of stay (9.68 days), suggesting more efficient patient throughput. These findings indicate that payment method may influence patient experience, treatment duration, and revenue generation.

Doctor Performance Analysis

5. Doctor performance analysis revealed significant variations in patient outcomes and revenue efficiency. Dr. Taylor achieved the highest patient satisfaction score (6.04) while also maintaining one of the shortest average lengths of stay (9.39 days), indicating strong clinical and patient-management performance. In contrast, Dr. Green generated the highest average revenue per patient (£13,968) but recorded the lowest patient satisfaction score (4.45). These findings suggest that higher revenue generation does not necessarily correspond with higher patient satisfaction and that physician performance should be evaluated using multiple measures rather than financial outcomes alone.

Severity vs Revenue

6. Severity-level analysis revealed that Medium (£1.69 million) and High (£1.68 million) severity patients generated the highest total revenue during the reporting period, while Critical patients generated the lowest (£1.50 million). Although Critical cases typically require more intensive care, they represented the smallest patient group (113 patients), suggesting that overall revenue performance was influenced more by patient volume than severity level alone.

Recommendations

1. Management should review factors contributing to extended patient stays, including discharge procedures, treatment workflows, and insurance-related administrative processes. Reducing unnecessary extended stays may improve operational efficiency without significantly impacting revenue generation.

2. Management should investigate the operational practices, treatment pathways, and resource utilization strategies employed in Sheffield, Cardiff, Glasgow, and Nottingham to identify factors contributing to higher revenue efficiency. Particular attention should be given to Cardiff, which demonstrated strong operational efficiency but suffered from low patient volume. Increasing patient admissions while maintaining current efficiency levels could significantly improve Cardiff's contribution to overall organizational revenue. Additionally, Cardiff's lower patient satisfaction score should be reviewed to ensure that operational efficiency is not negatively affecting patient experience and quality of care.

3. Management should strengthen preventive care, routine screening programs, and specialized treatment pathways for older patients, as they appear to be disproportionately represented among Critical cases. Early intervention strategies may help reduce disease severity and improve patient outcomes within this age group.

4. Management should review treatment workflows and administrative processes associated with Private payment plans to identify factors contributing to longer patient stays and lower satisfaction scores. Additionally, the practices associated with Employer-sponsored healthcare plans should be evaluated to determine whether their positive revenue and satisfaction outcomes can be replicated across other payment categories.

5. Management should evaluate the practices employed by high-performing physicians such as Dr. Taylor to identify approaches that contribute to shorter patient stays and higher satisfaction levels. Additionally, physicians generating strong financial outcomes but lower patient satisfaction should be supported through patient experience and communication improvement initiatives to ensure balanced performance across clinical, financial, and patient-centered metrics.

6. Management should continue monitoring resource allocation across all severity levels to ensure that high-severity cases receive appropriate care while maintaining operational efficiency. Additional analysis should be conducted to determine whether treatment costs and resource utilization are aligned with the complexity of Critical cases.